

National Audit of Pulmonary Hypertension

Local Action Plan - 10th Annual Report

Pg	Standard	Recommendation	Planned action
23	80 per cent of new referrals who have a diagnosis of idiopathic, heritable, anorexigen-induced or connective tissue disease associated PAH and are in WHO functional class II, III or IV, should commence PAH drug therapy within 12 weeks of receipt of their first referral.	PH centres who do not meet this standard should review the delays in their patient pathway.	Our performance against this standard has fallen from 87% in 2016-17 (standard met) and 86% in 2017-18 (standard met) to 79% in 2018-2019. We have identified the need to have a dedicated member of staff to track patients through the pathway to ensure that we achieve this target and have highlighted this as a priority for 2020. We are also undertaking a review of our service capacity and have appointed an additional medical consultant in October 2019 to increase our service capacity.
35	90 per cent of patients who undergo a pulmonary endarterectomy should wait less than four months from their diagnosis of chronic thromboembolic pulmonary hypertension.	NHS England and PH centres need to address the delay in undertaking pulmonary endarterectomy. This requires a joint effort since the patient pathway is complex.	We are working with the national pulmonary endarterectomy centre at the Royal Papworth Hospital to reduce time to surgery. We have also appointed 1 additional secretary to increase our capacity and reduce administrative delays for referral for initial assessment. We have a dedicated member of nursing staff monitoring the patient pathway and we are examining how to improve waiting time to coronary angiography for patients accepted for surgery.